

May 20, 2026

Senator Kristen Gonzalez, Chair
Internet and Technology Committee
Legislative Office Building
198 State Street, Room 817
Albany, NY 12247

Senator Daniel Stec, Ranking Member
Internet and Technology Committee
Legislative Office Building
188 State Street, Room 408
Albany, NY 12247

Senator Liz Krueger
Capitol Building
172 State Street, Room 416 CAP
Albany, NY 12247

Assemblymember Jeffrey Dinowitz, Chair
Standing Committee on Codes
Legislative Office Building, Room 632
Albany, NY 12248

Assemblymember Linda Rosenthal
Legislative Office Building, Room 943
Albany, NY 12248

RE: Letter in Opposition to New York Health Information Privacy Act (SB 9269 and AB 10357)

Dear Chair Gonzalez, Ranking Member Stec, Senator Krueger, Chair Dinowitz, and Assemblymember Rosenthal:

On behalf of the advertising industry, we write to oppose the New York Health Information Privacy Act proposed by New York SB 9269 and AB 10357 (collectively, “NYHIPA” or “the bill”).¹ We provide this letter to offer our non-exhaustive list of concerns about NYHIPA. The bill would hinder New Yorkers’ ability to access services and severely burden the operation of many businesses in the state. Moreover, the core concerns identified below have not materially changed from those that prompted Governor Hochul’s veto of substantially similar legislation last year. Accordingly, we ask you to decline to advance the bill as drafted out of the Senate Internet and Technology Committee and the Assembly Standing Committee on Codes (“Committees”).

As the nation’s leading advertising and marketing trade associations, we collectively represent thousands of companies across the country. These companies range from small businesses to household brands, advertising agencies, and technology providers. Our combined membership includes more than 2,000 companies that power the commercial Internet, which accounted for nearly 20 percent of total U.S. gross domestic product (“GDP”) in 2024.² By one estimate, approximately 16.8% of New York jobs in 2024 were related to the ad-subsidized Internet, a share projected to increase to 19.8% by 2029.³ Our group has more than a decade’s worth of hands-on experience it can bring to bear on matters related to consumer privacy and

¹ New York SB 9269 (2025-2026 session) located [here](#); New York AB 10357 (2025-2026 session) located [here](#) (collectively hereinafter, “NYHIPA”).

² S&P Global, THE ECONOMIC IMPACT OF ADVERTISING ON THE US ECONOMY, 2024-2029 at 4 (Aug. 2025), located at https://theadcoalition.com/wp-content/uploads/2025/08/TAC_SP-Global-Final-Report_August-2025.pdf.

³ *Id.* at 15-16.

controls. We would welcome the opportunity to engage with the Committees further on the points we discuss in this letter.

I. NYHIPA’s Definitions of “Regulated Health Information” and “Regulated Entity” are Overly Broad

NYHIPA’s terms, coupled with its overly broad definition of “regulated health information,” could unintentionally impede New Yorkers from receiving useful and relevant information about products and services they may desire. As defined, the term “regulated health information” would include *any information* that could possibly be related—however tangentially—to the health of a consumer, including data associated with persistent identifiers, inferred, or extrapolated data.⁴ The definition could be interpreted to include basic data points and encompass non-sensitive data, and therefore impact general advertising practices that do not involve identifying or targeting individuals based on actual health conditions. For example, a corner store and its advertising partners may collect information about a customer’s purchases to offer the customer discounts for products they may be interested in. The corner store may learn that the customer purchased a dandruff shampoo and want to offer the customer coupons for future purchases of similar items. However, under NYHIPA’s definition, doing so could be interpreted to be processing regulated health information in connection with a health status, even though the corner store and its partners are not using it in that way.

Offering a dandruff shampoo coupon at a local grocery store should not be treated the same as advertising that targets individuals based on specific, sensitive health diagnoses. Accordingly, NYHIPA should be updated to narrow the definition of the term “regulated health information.” We recommend that the definition of “regulated health information” be aligned with the majority of states that have passed legislation regulating “consumer health data,” such as the definitions of the term in the Connecticut, Maryland, and Nevada laws. In those state laws, “consumer health data” is personal information that is linked or reasonably capable of being linked to a consumer and that “*is used to identify*” the past, present or future health status of the consumer. This definition will help ensure that the definition is cabined to information an entity actually uses to identify a consumer’s health status, and that basic data points unrelated to a consumer’s actual physical or mental health are not unintentionally swept into the definition of the term. Our goal is to ensure that the legislation clearly distinguishes between use of data to identify sensitive, identified health conditions and ordinary advertising.

NYHIPA should also be updated to clarify that “regulated entity” refers exclusively to entities processing “regulated health information” of New York residents. We recommend refining the existing broad term defining a “regulated entity” to remove entities that control the processing of “regulated health information” about individuals “physically present” in New York.⁵ This update will help clarify that the bill applies exclusively to the “regulated health information” of New Yorkers and would help harmonize the bill’s terms with laws in other states.

⁴ NYHIPA at § 1120(2).

⁵ *Id.* at § 1120(4).

II. The Bill’s “Valid Authorization” Requirements Would Cause Consumer Frustration Without Providing Meaningful Privacy Protections

In part due to the NYHIPA’s broad definitions, the requirement to obtain “valid authorization”—a signed consent—every time a regulated entity processes regulated health information would inundate New Yorkers with an overwhelming number of consent requests for basic data processing activities.⁶ NYHIPA would permit a regulated entity to process regulated health information without a valid authorization for a narrow set of purposes, and would explicitly prohibit processing for “activities related to marketing, advertising, research and development, or providing products or services to third parties” absent valid authorization from the consumer.⁷ As a result, NYHIPA valid authorization requirements would significantly hinder the use of data to improve products or services, conduct research for the benefit of consumers, and apprise consumers of relevant offerings that may interest them – such as a coupon on shampoo – without obtaining signed consent for such activities. NYHIPA would also require consumer consent to be specific to the processing activity.⁸ NYHIPA is consequently likely to result in significant consent fatigue for New Yorkers instead of providing meaningful privacy protections for consumers. NYHIPA’s detrimental—and likely unintentional—consequences would hinder consumers from receiving significant benefits associated with routine and essential data practices while simultaneously placing overly burdensome requirements on regulated entities that process any information that could even vaguely be related to health.

III. Requiring Regulated Entities to Pass Consumer Rights Requests to Third Parties Could Conflict with Consumer Preferences

NYHIPA would require regulated entities to pass along deletion and correction requests to third parties and would require any third party that receives notice of such a request to delete all regulated health information associated with the consumer within 30 days of receipt of the request.⁹ NYHIPA should not require regulated entities to flow deletion requests through to third parties, because such actions may not align with consumers’ wishes. A consumer may desire, for instance, to delete the regulated health information maintained by a certain regulated entity but may not want other regulated entities—who would be “third parties” under the NYHIPA’s terms—to similarly be required to delete regulated health information. Such a requirement could impact products and services the consumer wishes to and expects to receive, as a deletion request served on one entity would be required to be cast to all other entities in the marketplace. The consumer may not intend to or want to serve a deletion request on regulated entities the individual knows, trusts, and wishes to continue to receive messaging from. Requiring deletion requests to be passed to other entities in the marketplace would create detrimental results the individual did not intend, desire, or expect.

⁶ *Id.* at § 1122.

⁷ *Id.* at § 1122(1)(b)(ii)(B).

⁸ *Id.* at §§ 1122(2)(a)(i), (v).

⁹ *Id.* at §§ 1123(2)(c), (d).

IV. NYHIPA's Unlimited Attorney General Rulemaking Authority Would Increase Variation With Other State Privacy Laws.

As drafted, NYHIPA would give the New York Attorney General (“AG”) broad authority to “promulgate such rules and regulations as are necessary to effectuate and enforce the provisions of this section.”¹⁰ Such authority would exacerbate the inconsistency that already exists amongst state health privacy laws by enabling the AG to issue rules that are out of step with privacy requirements in other states. Harmonization with existing privacy laws is essential for creating an environment where consumers in New York and other states have a consistent set of expectations, while minimizing compliance costs for businesses. Several sections of NYHIPA could be updated to harmonize the bill with similar laws in other states. For instance, NYHIPA should be revised to redefine “sell” to exempt transfers to service providers, include explicit exemptions to consumer rights and other obligations to ensure “regulated health information” may be maintained to prevent and detect fraud and respond to security incidents, and amend the data minimization provisions by changing “strictly necessary” to “reasonably necessary,” consistent with the approaches taken by other state laws.¹¹

Compliance costs associated with divergent privacy laws are also significant. To make the point: a regulatory impact assessment of the California Consumer Privacy Act of 2018 (“CCPA”) concluded that the initial compliance costs to California firms for the CCPA *alone* would be \$55 billion.¹² Additionally, a recent study on a proposed privacy bill in a different state found that the proposal would have generated a direct initial compliance cost of between \$6.2 billion to \$21 billion, and an ongoing annual compliance cost of between \$4.6 billion to \$12.7 billion for companies.¹³ Other studies confirm the staggering costs associated with different state privacy standards. One report found that state privacy laws could impose out-of-state costs of between \$98 billion and \$112 billion annually, with costs exceeding \$1 trillion dollars over a 10-year period and small businesses shouldering a significant portion of the compliance cost burden.¹⁴ New York should not enact a law that could add to this compliance burden for businesses. The legislature should remove NYHIPA’s rulemaking authority, or at the very least take steps to place more limitations on the rulemaking authority to foster more consistency across state regimes.

¹⁰ *Id.* at § 1127(6).

¹¹ *Id.* at §§ 1120(5), (2); § 1122(1)(b). See Wash. Rev. Code § 19.373.010(26)(b)(ii); Conn. Gen. Stat. § 42-524 (f).

¹² See State of California Department of Justice Office of the Attorney General, *Standardized Regulatory Impact Assessment: California Consumer Privacy Act of 2018 Regulations* at 11 (Aug. 2019), located at https://dof.ca.gov/media/docs/forecasting/economics/major-regulations/major-regulations-table/CCPA_Regulations-SRIA-DOF.pdf.

¹³ See Florida Tax Watch, *Who Knows What? An Independent Analysis of the Potential Effects of Consumer Data Privacy Legislation in Florida* at 2 (Oct. 2021), located at <https://floridatxwatch.org/DesktopModules/EasyDNNNews/DocumentDownload.ashx?portalid=210&moduleid=34407&articleid=19090&documentid=986>.

¹⁴ Daniel Castro, Luke Dascoli, and Gillian Diebold, *The Looming Cost of a Patchwork of State Privacy Laws* (Jan. 24, 2022), located at <https://itif.org/publications/2022/01/24/looming-cost-patchwork-state-privacy-laws> (finding that small businesses would bear approximately \$20-23 billion of the out-of-state cost burden associated with state privacy law compliance annually).

V. Clarify the Bill Does Not Include a Private Right of Action.

To ensure that enforcement remains within the Attorney General’s purview and to avoid any confusion regarding who can take legal action, NYHIPA should explicitly state in the bill that it does not grant a private right of action. Without an explicit statement, potential legal ambiguity could create confusion among businesses and consumers.

VI. The Bill’s Compressed Implementation Period Does Not Consider the Operational Burden of Compliance.

NYHIPA’s abbreviated effective date of six months raises significant concerns for businesses that would be subject to sweeping new compliance obligations on an accelerated timeline.¹⁵ Previously introduced versions of NYHIPA included a one-year effective date provision.¹⁶ Given the expansive scope of the bill and the ambiguity surrounding key definitions, businesses across multiple sectors would require substantial time to stand up a compliance framework and a compressed implementation period would create considerable operational burdens and costs, particularly for small and mid-sized businesses that lack extensive compliance infrastructure, while increasing the risk of inadvertent noncompliance.

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¹⁵ NY HIPA § 3.

¹⁶ See New York S. 929 (2025 sess.) at § 3, located [here](#).



We and our members support protecting consumer privacy. We believe, however, that NYHIPA takes an overly broad approach to the collection, use, and disclosure of any data that could possibly be related to or indicative of health. We therefore respectfully ask that you carefully consider these negative impacts of the NYHIPA and decline to advance the bill as drafted.

Thank you in advance for your consideration of this letter.

Sincerely,

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